CARDIOCARE AI

Cardiovascular screening • Demonstration document

CLINICAL INVESTIGATIONS
Sample report for document-upload / ML pipeline testing

DOCUMENT ID: DEMO-LAB-042
STATUS: SIMULATED

SYNTHETIC / FICTIONAL PATIENT DATA — NOT A REAL MEDICAL RECORD

This document contains invented values designed to exercise file upload, text extraction, field parsing, and ML input mapping. Do not use it to make clinical decisions or as evidence of a real patient's condition.

PATIENT / SPECIMEN DETAILS	DETAIL	PATIENT / SPECIMEN DETAILS	DETAIL
Patient name	Demo Patient (fictional)	Patient ID	DEMO-CC-042
Date of birth / age	Not supplied / 58 years (fictional)	Sex	Not specified
Collection date/time	19 Oct 2026, 09:15 IST (simulated)	Report date/time	19 Oct 2026, 10:05 IST (simulated)
Ordering service	Cardiology — demo workflow	Specimen	Synthetic values; no physical specimen

Clinical context entered for this test case

Fictional symptom pattern: chest pressure/tightness, exertional breathlessness, palpitations, and unusual fatigue. Chest discomfort is marked substernal, provoked by exertion/stress, relieved by rest within 10 minutes, described as heaviness/pressure/tightness, with radiation to the left arm. Congestive-sign selections: dyspnea at rest = no; orthopnea = yes; paroxysmal nocturnal dyspnea = yes.

A. Cardiac biomarkers

Test	Result	Units	Illustrative reference interval / note	Flag
High-sensitivity cardiac troponin I (hs-cTnI)	38	ng/L	Example lab 99th percentile URL varies by assay/sex	Above example URL*
Repeat hs-cTnI (1 hour)	46	ng/L	Interpret serial change using validated assay pathway	Rise in sample values
NT-proBNP	420	pg/mL	Interpret with age, setting, renal function and clinical context	Context-dependent

*The troponin values above are intentionally fictional and are not linked to any real assay or patient. Real interpretation requires the actual laboratory's assay-specific reference limit and clinician assessment.

B. Electrocardiogram (ECG)

Study	Simulated report
12-lead resting ECG	Sinus tachycardia, ventricular rate approximately 104 bpm. Nonspecific ST-T wave changes. No STEMI conclusion is assigned in this synthetic example. Correlation with symptoms and clinician interpretation required.
Acquisition / review	Demonstration text only; no ECG waveform was recorded or interpreted.

SYNTHETIC TEST DATA

C. General laboratory panel

Test	Result	Units	Illustrative reference interval	Flag
Hemoglobin	13.6	g/dL	12.0–16.0 (example; lab-specific)	Within example range
White blood cell count	8.2	10■/L	4.0–11.0	Within example range
Platelet count	248	10■/L	150–400	Within example range
Sodium	139	mmol/L	135–145	Within example range
Potassium	4.2	mmol/L	3.5–5.1	Within example range
Creatinine	1.0	mg/dL	Approx. 0.6–1.3; lab-specific	Within example range
Estimated GFR	86	mL/min/1.73 m²	Interpret with age and clinical context	Illustrative
Fasting glucose	108	mg/dL	Fasting range depends on guideline/lab	Slightly elevated example
HbA1c	5.8	%	Commonly <5.7% normal; criteria vary	Borderline example
Total cholesterol	212	mg/dL	Desirable commonly <200	Elevated example
LDL cholesterol	138	mg/dL	Targets depend on cardiovascular risk	Elevated example
HDL cholesterol	43	mg/dL	Interpret by sex and risk context	Illustrative
Triglycerides	156	mg/dL	Commonly <150 fasting	Borderline example

D. Transthoracic echocardiogram (TTE)

Parameter	Simulated finding
Left ventricular ejection fraction (LVEF)	55% (illustrative)
Regional wall motion	No definite regional wall motion abnormality stated in this sample text
Valves / chambers / pericardium	No detailed findings provided; sample report intentionally incomplete

E. Sample screening summary (not a diagnosis)

Application rule output	Example value	Interpretation note
Angina classification	Typical angina	Mirrors the classification text supplied for this software test case.
Congestive signs score	3 points	Mirrors the application output; scoring logic must be validated separately.
Priority tier	Emergency / immediate EMS	Expected red-flag path for the supplied symptom pattern; this PDF does not execute the model.

SYNTHETIC TEST DATA

Testing guidance

Upload this PDF only into a development/test environment. Verify separately that the application extracts values (for example, hs-cTnI, NT-proBNP, BP, heart rate, oxygen saturation, and ECG text), preserves units and flags, and does not silently convert missing values to zero. Compare parsed fields with the source text before evaluating model predictions. A single document cannot establish model accuracy; use a labelled, held-out test set for that.

Emergency safety statement

For real current chest pressure or concerning associated symptoms, seek emergency medical help immediately. Do not delay care to upload a document or run a screening model.

End of synthetic report • No real specimen, ECG tracing, or patient encounter exists.